

Intra-articular knee

NOVA-MSC FLEX + NOVA-EXOS CORE · physician reference protocol

PER KNEE 2 cc total FLEX 1 cc + EXOS CORE 1 cc	BILATERAL 4 cc total 2 cc per joint	NEEDLE 22–25 G 1.5 in, longer if indicated	OBSERVATION 10–15 min post-treatment
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A potential approach for intra-articular administration in patients presenting with symptomatic knee degeneration, osteoarthritis, chronic knee pain, meniscal degeneration, or cartilage wear.

Indications

- Mild-to-moderate osteoarthritis
- Chronic knee pain
- Degenerative meniscal pathology
- Cartilage wear without complete joint collapse

Contraindications

- Active infection or septic arthritis
- Uncontrolled coagulopathy
- Known product hypersensitivity
- Intra-articular corticosteroid within four weeks

Procedure

PRE-PROCEDURE

1. Review patient history and available imaging.
2. Obtain informed consent.
3. Verify product preparation and integrity.
4. Position supine, knee flexed ~90°.
5. Prepare skin with chlorhexidine or povidone-iodine.

POST-PROCEDURE — PATIENT INSTRUCTIONS

- Avoid NSAIDs 48–72 h when clinically appropriate
- Avoid alcohol 48–72 h
- Limit strenuous lower-extremity activity 2–3 days
- Cold therapy as needed; resume rehab as tolerated

INJECTION

1. Aspirate effusion if present.
2. Confirm intra-articular placement.
3. Slowly administer 1 cc NOVA-MSC FLEX and 1 cc NOVA-EXOS CORE.
4. Apply sterile dressing after needle withdrawal.

Preferred: anterolateral or anteromedial approach. Alternative: superolateral with knee in extension.

FOLLOW-UP & OUTCOME MEASURES

Clinical assessment at **4–6 weeks**. Consider repeat treatment or adjunctive therapy at **8–12 weeks** based on provider assessment and response.

VAS PAIN

WOMAC

KOOS

FUNCTIONAL STATUS

ACTIVITY LEVEL

REGULATORY

CelluNOVA products are supplied for use by licensed healthcare professionals. CelluNOVA does not direct, prescribe, or control the practice of medicine, and nothing here guarantees a clinical outcome.

Today you received an injection of cell and exosome products into your knee joint. The products used are made from donated birth tissue and are not FDA-approved treatments. This page tells you what is normal in the days ahead, how to care for yourself, and when to call us.

DATE OF TREATMENT

SITE(S) TREATED

NEXT VISIT

WHAT TO EXPECT

Changes are gradual — do not judge the result in the first days

FIRST 72 HOURS

Soreness, warmth, or mild swelling in the knee is common and usually settles in two to three days. A short flare of your usual pain can happen.

WEEKS 1–4

Ease back into normal activity as comfort allows and restart physical therapy or home exercises as advised.

WEEKS 4–12

This is when progress is assessed. Keep your follow-up visit so we can measure how the knee is responding.

DO

- Take it easy the rest of today; gentle walking is fine
- Use cold packs 10–15 minutes at a time for comfort
- Take acetaminophen (Tylenol) for soreness if needed
- Resume physical therapy or home exercises as advised

AVOID

- Anti-inflammatory pain relievers (ibuprofen, naproxen, aspirin for pain) for 72 hours
- Alcohol for 72 hours
- Squatting, impact, or strenuous lower-body exercise for 2–3 days
- Hot tubs or soaking the injection site for 24 hours

CALL US RIGHT AWAY IF

For an emergency, call 911 first

- The knee becomes increasingly hot, red, and swollen rather than better
- Calf pain or swelling, or shortness of breath

- Fever above 101 °F or shaking chills
- Severe pain not controlled by acetaminophen and cold packs

CLINIC PHONE

AFTER-HOURS PHONE

NOTES FROM YOUR VISIT